

Our Ref: BGM/bp (T) // [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

INTERSTITIAL LUNG DISEASE

[PROVIDER-NAME] and Honorary Senior Clinical Lecturer

[PROVIDER-NAME] and Honorary Associate Professor in Medicine

[PROVIDER-NAME]

[PROVIDER-NAME]

Division B

Care Group - Emergency & Specialist Medicine Respiratory Medicine

Minerva House, 2nd Floor, Mailpoint

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Stage III sarcoidosis

Inflammatory bowel disease

Persistent fatigue

I was pleased to see Mr [NAME] wife and granddaughter today. He looked extremely well but continued to be plagued by fatigue which is impacting on his work although I gather he is still working 12-hours a week. His bowel function is good. His lung function tests show maintenance in the improvement shown previously demonstrated in the past with the addition of steroids and Azathioprine. Chest X-ray in March definitely shows improvement both in the adenopathy in the hilar areas but also in the parenchymal changes. All his bloods are okay recently.

The side effects to steroids include weight gain and also early cataract formation.

I think it is time to try and taper the dose of his immunosuppression and I have suggested he reduce his Azathioprine dose to 25mg once a day and the Prednisolone dose to 5mg once a day.

He is willing to give this a go. I suspect this will not improve his fatigue symptoms but certainly an increase in dose did not help either.

He is up for a seasonal flu vaccine later in the year.

We shall keep him under review at four-month intervals and when I see him in March next year repeat his chest X-ray as well as the lung function.

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Respiratory Physician and

Honorary Associate Professor in Medicine